

would ever love me.

Therapist: Do you agree, Sonia? (*She nods, looking at the floor.*) Okay, so let the critical part know that, yes, there was a young girl who had all that responsibility and no grown-up help. Her mother was ill, and she tried to keep her little brother safe, but she was too young. Who was taking care of her? She was vulnerable too. She was alone and scared, and she had a critic beating her up inside. (*pausing*) Can you see her in that foster home?

Sonia: I see her... She's sad.

This is the exile.

Therapist: Is she aware of you? (*Sonia nods.*) Let her know you're here now. Would she like your help?

Sonia: Yes.

Therapist: Tell her we'll come back to her soon. First, we have to check in with the parts who drank and smoked and so forth to get their permission. Can you see them too?

Sonia: I see the fourteen-year-old. She needed Marty and the booze.

This is a firefighter part.

Therapist: Do you get that she needed Marty and the booze? (*Sonia nods.*) Would she like your help too?

Sonia: Yes.

Therapist: Okay, great. We'll come back to her.



Since Sonia was convinced that she was bad as a child (and was still bad now), it was clear that her judging, critical managers were still present. At the same time, she felt unlovable, so her exiles were also in her consciousness. This is a formula for firefighter intervention that can lead to a recurrence of compulsive behavior. While many clients like Sonia can readily, even dispassionately, recount a devastating family history, we should not mistake that dispassion for calm and self-compassion. Their protectors have no idea how to meet the needs of traumatized, abandoned parts. In IFS, we assert that it's never too late to heal attachment wounds and bring exiles to a safer place. We make sure firefighters have a chance to explain their good intentions, and we validate the legitimate cravings of exiled parts for comfort, warmth, adult protection, and safety.

ASKING ABOUT OUTSIDE INFLUENCES

Many clients trying to control their addictive processes are subject to external pressure from loved ones. Their behavior is chaotic, they habitually disappoint others, they lack follow-through, and they have legal and medical problems. All this recruits the manager parts of friends and family, and also of the health care professionals who cycle through their life trying to help. All of these well-intentioned

people (including us!) explain, shame, threaten, manipulate, beg, and nail down contracts in which the client agrees to minimize or stop the addictive behavior. However, this external pressure won't end the chaos, stop the cycle of self-destruction, or effect a cure. In contrast, it will motivate firefighters to resist intervention and act out more stridently.

Of course, some clients have family members or partners who are more troubled, chaotic, and self-destructive than they are, in which case therapy needs to help the client create new boundaries and support parentified or desperate young parts. This is particularly true in families that have little or no apparent interest in recovery, where the client faces wrath, derision, or exile if they choose a more settled, manager-led life. While distressed families deserve an equal measure of compassion, the client has an obligation to free their parentified young rescuers from an impossible task.

Our job is to help clients explore their relational network, including outside pressures and obligations, so we can clarify external polarities that activate and impact their compulsive behaviors. How does the client spend their day? Whom do they see? What are those relationships like? What we often find is that a client's firefighters are polarized with the manager parts of a partner, a parent, or some authority figure, which bolsters their determination and hinders the client's access to manager parts.

QUESTIONS FOR CLIENTS ABOUT OUTSIDE INFLUENCES

- How much does _____ know about your substance use (or bulimia, gambling, or other patterns of compulsive behavior)?
- How much information do you feel safe sharing with them?
- What do you *not* want to share with them?
- What are you afraid would happen if they knew everything?
- Have their opinions or feelings about this behavior changed over time?
- Do you and _____ argue about this compulsive behavior?
- Does _____ have a compulsive habit that worries you?
- When _____ complains about your behavior, how do you respond?
- How does your response impact the relationship?
- If your relationship with _____ was good, what would it look like?
- Do you and _____ engage in this behavior together? If yes, what is that like for you?
- If you stopped this compulsion, how would that impact _____?
- If _____ stopped their compulsion, how would that impact you?
- Have you shared your vision or plan about your addictive behavior with _____?

- Do you know what plan or vision _____ has for their compulsive behavior?
- Do you feel you can speak honestly with _____ about your plan?
- Do you have fears about the relationship ending if you make changes?

BEFRIENDING MANAGERS

If you aren't already engaging in shuttle diplomacy between firefighters and managers as you complete the firefighter history review, it's important to go back to the manager team. Negotiating with these more socially astute parts may seem counterintuitive, but their shaming is what motivates firefighters to engage in compulsive practices. Therefore, if managers calm down, firefighters are much more likely